

SAMPLE — REPLACE WITH THIS CYCLE BEFORE SENDING TO HQ

THE ASK · FIRST 30 SECONDS

HQ SHOULD DO THIS

Concentrate the next two weeks on initiation barriers in priority private accounts. Do not treat call volume as the progress metric.

WHY NOW

Private-sector PSP expiry (end-2026) is already changing close conversations. Formulary packaging needs field themes, not visit counts, while the evidence window is still open.

RECOMMENDATION

Do: report 3 conversion signals mapped to uptake / formulary / compliance. Do not: send trip logs, named HCPs, or CRM dumps.

FORMULARY THIS CYCLE

No change to dossier — themes usable, no new unlock

PSP IN PLAY

Base Case + Private Sector (expires end-2026)

COMPLIANCE

Green · PV-only lexicon · ET/MF reactive-only

DECISION NEEDED BY

[YYYY-MM-DD]

OWNER

PM, with GM for HQ send

GOES INTO

Standing biweekly HQ update

FIVE-SECOND SCORECARD

Initiation, not awareness, is the stall.

UPTAKE VS PLAN

AMBER

Awareness holds; initiation is slow.

Definition: private uptake vs agreed HQ plan — not vs last week's call count.

Vs last cycle: unchanged conversion quality.

FORMULARY TRAJECTORY

AMBER

Field themes are usable; dossier not advanced this cycle.

Definition: whether this cycle strengthened, delayed, or did not change HADF packaging.

Vs last cycle: no new evidence gap; no new unlock.

COMPLIANCE

GREEN

No uncleared claims. ET/MF stayed reactive-only.

Definition: materials cleared; no open UMAO/HKAPI issues; lexicon intact.

Vs last cycle: clean. Keep raw notes in the vault.

What repeated in the field — not who we visited

Affordability is understood; initiation is not.

OBSERVED

Priority private accounts can repeat PSP logic, then still do not start.

SO WHAT FOR HQ

Uptake risk sits after the affordability pitch, not before it.

IMPLICATION

Next cycle: onboarding friction, not more awareness meetings.

Price vs traditional IFN is the #1 close-blocker.

OBSERVED

Same objection across accounts: why pay a premium vs off-label IFN.

SO WHAT FOR HQ

Differentiation must stay inside approved PV evidence and dosing convenience.

IMPLICATION

No IFN-to-IFN superiority claim. Standing lexicon response only.

Two PSP paths are creating two close motions.

OBSERVED

Base Case vs Private Sector (expires end-2026) are used as different close paths.

SO WHAT FOR HQ

Expiry is now a sequencing issue for HQ, not a slogan for the field.

IMPLICATION

Do not informal-tweak terms. Confirm eligibility before quoting.

CONVERSION, NOT COVERAGE

Where field time dies

ACCOUNT MOTION (QUALITATIVE)



Widths are illustrative of where accounts stall. Replace with your qualitative read — do not invent counts.

RANKED BARRIERS (MAX 4)

#	BARRIER	HITS	VS LAST CYCLE
1	Initiation / onboarding after PSP explanation	Uptake	New this month as the pattern
2	Price vs traditional IFN — lexicon-bound answer only	Uptake + compliance	Unchanged
3	Private Sector PSP clock changing urgency unevenly	Access / PSP	Rising
4	[Fourth barrier or delete row]	—	—

Moved this cycle: Stopped spreading coverage. Next two weeks: initiation checklist on the named priority-account list (titles only, no HCP names on this slide).

Options, risks, and the HQ ask

NOT RECOMMENDED

A. Keep activity reports

Visit counts and trip notes. Easy to write; HQ cannot see the thesis.

RECOMMENDED

B. Signal → implication → ask

This pack. Three patterns, RAG vs plan, one decision date.

NOT RECOMMENDED

C. CRM dump

Looks rigorous, usually unreadable, often too sensitive to forward.

PRE-MORTEM

If this fails, it is most likely because: Someone still scores the team on call volume, so activity theatre returns.

SHAREABLE-PACK RISK

A named KOL or patient-adjacent story leaks into a forwardable deck.

NEXT ACTIONS

ACTION	OWNER (ROLE)	WHEN
Rewrite last trip report into this 5-slide pack; strip names	PM	This week
Lock RAG definitions with GM (uptake / formulary / compliance)	PM + GM	Next HQ cycle
Compliance pass: no ET/MF promotion, no uncleared claims	PM + Legal/Compliance	Before send